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## *FOOD, NUTRITION AND HEALTH POLICY*

Report on a Workshop supported by the  
United Kingdom Government  
and Know-How Fund

Moscow, Russian Federation  
20–24 April 1998

## EUROPEAN HEALTH21 TARGET 11

### HEALTHIER LIVING

By the year 2015, people across society should have adopted healthier patterns of living

*(Adopted by the WHO Regional Committee for Europe at its forty-eighth session, Copenhagen, September 1998)*

### ABSTRACT

The Know-How Fund (United Kingdom), Health Promotion Wales, the Institute of Nutrition of the National Academy of Medical Sciences, Moscow, the Centre for Preventive Medicine, Moscow and the WHO Regional Office for Europe jointly organized a Workshop on Health, Nutrition and Food Policy in the Russian Federation. This was a follow-up to the preliminary workshop held in October 1997 (see WHO Regional Office for Europe document EUR/RUS/LVNG 02 01 09). This Workshop addressed issues related to multisectoral activities at a national and regional level, including the agriculture sector, the food industry and consumers. The participants included professionals from Arkhangelsk, Chelyabinsk, Krasnodar, northern Russia and Moscow and from Finland and the United Kingdom. The issues discussed during the three-day Workshop, including the impact of nutrient recommendations and dietary guidelines on agriculture production and consumer food intake, convinced the health professionals attending of the need to work with other sectors when developing nutrition and health policy. The participants gained insight into the diverse perspectives of food and nutrition policy necessary to help tackle the public health challenges of the Russian Federation. It is anticipated that this Workshop will be followed by the development and implementation of food and nutrition policies at regional level.

### Keywords

NUTRITION POLICY  
HEALTH POLICY  
INTERSECTORAL COOPERATION  
SUSTAINABILITY  
RUSSIAN FEDERATION

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Thanks are also due to the health professionals working in the regions of the Russian Federation who participated, especially those who drafted preparatory regional papers beforehand and gave presentations during the Workshop.

Finally, the enthusiastic participation and valued contributions of all the participants, without whom the Workshop would not have been possible, is deeply appreciated.

## CONTENTS

|                                                              | <i>Page</i> |
|--------------------------------------------------------------|-------------|
| Foreword.....                                                | 1           |
| Summary.....                                                 | 2           |
| Background and development of the project.....               | 3           |
| The purpose of this project .....                            | 4           |
| Expected outcomes.....                                       | 5           |
| Achievements of the project.....                             | 6           |
| Partnerships and sustainable developments.....               | 9           |
| Roles of partners.....                                       | 10          |
| Health Promotion Wales .....                                 | 10          |
| World Health Organization .....                              | 10          |
| National Centre for Preventive Medicine.....                 | 11          |
| Institute of Nutrition.....                                  | 11          |
| Complementarity of the project.....                          | 12          |
| Lessons learnt and problems experienced .....                | 12          |
| Other information.....                                       | 13          |
| Developments in the health sector.....                       | 14          |
| The way forward.....                                         | 14          |
| Annex 1 Programme.....                                       | 16          |
| Annex 2 Summary of feedback from working groups .....        | 18          |
| Annex 3 Summary of discussions .....                         | 21          |
| Annex 4 Guidelines for regional nutritional policies .....   | 23          |
| Annex 5 Evaluations and correspondence from partners.....    | 25          |
| Annex 6 Participants .....                                   | 31          |
| Annex 7 Resource documents provided during the Workshop..... | 32          |

## Foreword

This report summarizes the outcomes of a Workshop on Food, Nutrition and Health Policies in the Russian Federation, supported by the Know-How Fund Health Sector Small Partnerships Scheme in the United Kingdom. It was jointly organized by four partners including the National Centre for Preventive Medicine and the Institute of Nutrition, both in Moscow, Health Promotion Wales and the WHO Regional Office for Europe in Copenhagen.

The Workshop was held from 21 to 23 April 1998 at the Centre for Preventive Medicine in Moscow, with briefing, debriefing, planning and evaluation meetings between the partners on 20 and 24 April. The Centre for Preventive Medicine was the main coordinator in the Russian Federation and provided additional administrative and professional support.

The main purpose of the Workshop was to develop an intersectoral approach in the development of food, nutrition and health policies at national and regional levels.

Preparatory papers were developed and submitted by the participating regions and a number of key presentations were made, which helped to inform the participants why food and nutrition policy is needed to protect health in the Russian Federation. Some options were presented for taking this further. The Workshop drew together Russian experts from the public and private sectors and consumers' organizations and provided an insight into their different views on food, nutrition and health. International experts from Finland and the United Kingdom were able to make special contributions based on their experience in developing and implementing food and nutrition policies to promote health in their countries.

The short plenary sessions were followed by working groups. During the working groups the participants were asked to put forward suggestions as to how a food and nutrition policy could be formulated in the context of the disease patterns and health needs in their regions. A considerable amount of information and experience was shared over the very short period of the Workshop. At the end of the three days, there was a clear indication that participants were developing a vision on how to move forward.

The overall feedback was extremely positive and the dynamic group work clearly demonstrated that the participants had the motivation, commitment and some of the skills needed to accomplish this major undertaking over the next few years.

Helen Howson  
Health Promotion Wales  
May 1998

## Summary

Since the mid-1990s some serious efforts have been made to build a framework for health promotion and disease prevention at national and regional levels in the Russian Federation. There has been ongoing discussion about the reasons for the shortening life expectancy, which in men has fallen to less than 60 years of age. The reasons are complex and factors such as stress, inequalities, increased alcohol consumption and food intake have all been suggested as underlying causes. The major burden on health care services is from the morbidity levels due to the high prevalence of chronic noncommunicable diseases such as cardiovascular and cerebrovascular diseases, diabetes mellitus, hypertension and cancers. From experience in the west it is known that these are largely preventable and so greater efforts must be made to build a framework for health promotion and disease prevention in the Russian Federation.

As yet few links have been made between general eating habits and healthy public policy in the Russian Federation. There appears to be no clear general understanding of international nutrient reference values and dietary guidelines. Moreover, these international guidelines have not yet been compared with the different eating patterns in different parts of the Russian Federation. In view of the lack of scientific data, many health professionals believe that the major diet-related health problems in the Russian Federation are due to deficiency of nutrients such as proteins, vitamins and minerals. Until the relationship between health and diet is clarified by presentations of data by Russians on food intake and nutritional status, it will continue to be difficult for the majority of health professionals to change their entrenched beliefs.

A national nutrition policy is being considered by the National Centre for Preventive Medicine and will be submitted to the parliament. However, this national policy has not been developed with a view to implementation in the different regions of the Russian Federation, and there has been no regional input into the policy development process. Experience in the west shows that policies are unlikely to be implemented if stakeholders are not involved in the development process and a sense of ownership is not fostered from the start. Therefore, using the national nutrition policy as a basis, there is a need to work with regions to help them identify regional diet-related health problems, collect available data on food intake, nutritional status, and mortality and morbidity patterns and produce a policy framework which can be used to help implement the national food, nutrition and health policy in different regions. There has also been little attempt to link nutrition to the wider context of healthy public policy.

The purpose of the workshop was to bring together participants from different sectors and regions to explore health problems and to consider the necessary steps in developing a regional framework for intersectoral food, nutrition and health policies that can be translated into different regional needs and eventually implemented at regional level. The participants' list is at Annex 6.

The Workshop was opened by the Know-How partners, who gave welcome speeches and expressed their appreciation of the support received from the Know-How fund. This was followed by 16 brief plenary presentations to provide the background and rationale for the Workshop. Participants then divided into working groups to discuss the development of guidelines for regional intersectoral food, nutrition and health policies, including an outline of the possible content and the process that would be needed to develop a regional framework for policy development.

Each group presented its recommendations to the Workshop during the feedback session, which was an innovation in the Russian way of working. The participants are to be congratulated for performing so well in this unfamiliar democratic way and, indeed, this is one of the achievements of the workshop. It was agreed that the regions would carry out needs assessments from June to August, and a follow-up meeting to assess progress would be held at the beginning of October, probably in Chelyabinsk. It was hoped that the first draft of a policy framework would be available in December 1998.

Lessons learnt from international experience highlight the fact that health promotion based only on the provision of nutrition education is unlikely to be effective unless people also have access to healthy and affordable food. This will be one of the major challenges facing the regions when developing their food, nutrition and health policies. Food was heavily subsidized in the former USSR and everyone could afford large quantities of meat and milk. As the Russian Federation enters the market economy this will change. Health professionals will therefore have to be prepared to cope with the dietary changes resulting from fiscal and other policy changes, and the likely impact on health. To do so, they will need new skills and tools, an essential constituent of which are sound and scientifically-based food, nutrition and health policies. It is hoped that the foresightedness of the Know-How Fund in supporting this project will continue. The partners look forward to receiving feedback in order to build on the experience gained so far in a sustainable way.

## **Background and development of the project**

Over the last few years serious efforts have been made to build a framework for health promotion and disease prevention in the Russian Federation at national and regional levels. Two documents *Towards a healthy Russia, policy for health promotion and disease prevention: focus of major noncommunicable diseases* and *Policy and strategies for the prevention of cardiovascular and other noncommunicable diseases within the context of public health reforms in Russia*, described the health situation and the main strategies for developing and implementing national and regional policies for disease prevention and health promotion. Questions about what kind of food and nutrition policies are needed in the Russian Federation have also been asked at national and international meetings. However, as yet no clear links have been established between general eating habits and healthy public policy, and national food and nutrition policies have not been developed with a view to implementation in the regions of the Russian Federation.

The Russian Federation presents a number of interesting challenges for developing health and social policy. While the major policy developments in the rest of Europe are concerned with adding "life to years", "health to life" and "years to life", the major concern in the newly independent Russian Federation is the shortening life expectancy. This is the first time such a trend has occurred in a major industrial nation. The concern in the Russian Federation is thus regaining "life years lost". There has been much discussion about the reasons for the shortening life expectancy. The reasons are complex and have been related to factors such as inequalities, increased alcohol consumption and food consumption/dietary intake (1). Although there are health problems from acute diseases such as tuberculosis and other infectious diseases, the major health problems are those of chronic lifestyle-related diseases.

The Russian Federation comprises over one half of the former USSR's population. Its health policy in recent years has been the subject of considerable review and policy appraisal (e.g. WHO 1992 (2), State Research Centre of Preventive Medicine 1994 (3), International

Conference on Healthy Nutrition Policies in Russia 1997 (4)). Food availability is estimated to be more than adequate: the FAO, for instance, estimates that there was an average availability of 2990 Kcal per person per day in the period 1993–1995 (5).

Fresh meats and dairy products were consumed in vast quantities at affordable prices in the former USSR, aided by a central planning system which coordinated and assured distribution to the population. In a free market economy, safeguards for food supply and cost no longer exist. Despite this realignment of economic priorities, the fact remains that the average Russian citizen is probably suffering from excess intake of fat and protein, rather than a deficiency. The 55% obesity rate is an indication of this worrying trend (6). The high fat and protein intake may partly be explained by the standards established in the former USSR. These standards or “norms”, if interpreted literally by food producers, would lead to agriculture policies which encouraged the increased production of meat and other animal products. The recommended daily intake (RDI) for protein was set well above those in Europe and North America.

There still appears to be over-consumption of fat and protein in the form of meat products, despite the higher prices of these products, whereas the consumption of fresh fruit and vegetables appears to be very low even though they are cheaper than meat and meat products. There is a legacy from the original high RDI that perpetuates the belief that high intakes of fat and protein are necessary for maintaining health. This has resulted in many health professionals believing that the Russian population is suffering from under-nutrition (such as protein deficiency) rather than over-nutrition (7). This excess intake combined with low levels of physical activity contribute to the high obesity levels (8).

The argument for a food and nutrition policy is that it should incorporate broader social issues such as the environment, employment and access to food, and not just education about nutrition. Issues include debates about food security, the likely impact of trade regulations and the need for international food safety rules. In particular, food and nutrition policy has to take account of the social and economic changes which the Russian Federation is going through. Lessons learnt from international experience shows us that health promotion based only on the provision of education is unlikely to be effective unless people also have access to healthy and affordable food.

There is a need to put food and nutrition policy within the context of the health promotion and disease prevention activities of the Russian Federation. This requires the establishment of intersectoral links and cooperation between partners in other sectors, not just the medical authorities and public health workers. Taking into account the process of decentralization, the increased possibilities for decision-making and the availability of resources in the regions, an initiative is needed to investigate how nutrition policy could be developed and implemented at the regional level.

## **The purpose of this project**

A preliminary workshop had been held in October 1997 for representatives of the health sector. This had addressed specific issues including:

- the role of the Ministry of Health in relation to food
- current dietary physiological norms in the Russian Federation
- current disease patterns and possible means of prevention
- current eating patterns and dietary guidelines for the Russian Federation



- intersectoral collaboration
- education and teaching of food culture in the Russian Federation
- consumer concerns related to food
- community interventions
- imported foods and labelling of foods.

Following this workshop, one of the main objectives of the project had been to secure intersectoral collaboration. A Know-How Fund Workshop was accordingly organized on 21–23 April 1998 at the Centre for Preventive Nutrition in Moscow, with the overall purposes of encouraging partnerships and collaboration across health and other sectors in the development of national and regional food, nutrition and health strategies and strengthening the role of the health sector in health and human development. Participants explored current health policies and considered the steps needed to develop an intersectoral food, nutrition and health policy.

The immediate aim of the Workshop – to develop a model framework for food, nutrition and health policy development which could be applied at both national and regional levels – and its objectives had been revised and updated in January 1998 in line with current developments and progress being made at national and local levels. The objectives were to:

- encourage ownership and participation in developing a policy framework with a range of professional and community representatives at national and regional level;
- identify the development process necessary for building a policy and action plan;
- identify the key elements of a replicable framework for extended use nationally and in other regions; and
- consider briefly the action needed to translate policy into practice.

The main participants were key policy- and decision-makers from the health sector. Five regions were represented, including Chelyabinsk (5 participants), Electrostal (3), Tyumen (3), Krasnodar (1) and Arkhangelsk (1). The Know-How partners in Moscow were represented by 10 health professionals from the Centre of Preventive Medicine and the National Centre for Preventive Medicine. Three representatives from the International Confederation of Consumers (KONFOP) attended along with one representative each from the Anti-Monopolies Commission and the Ministries of Health, Education, and Science and Technology. Two representatives came from the private company Soya Ltd., one from WHO, and one each from Finland, Scotland and Wales, making 36 participants in total. Simultaneous interpretation was provided and Russian interpreters were hired from Moscow.

### ***Expected outcomes***

The following were the main outcomes anticipated from the Workshop:

- to strengthen the national and international role of the Government in the new food economy
- to strengthen the national alliance between food safety and nutrition
- to foster consumer confidence
- to consider developing dietary guidelines that are compatible with public health
- to identify strategies to develop and implement dietary guidelines.

Formal presentations were made in Russian and English covering a wide variety of issues from consumerism to nutrition policy development (see programme in Annex 1). These provided the basis for more detailed discussions in groups on the development of a framework for the policy.

The feedback from these groups (Annex 2) led to further debate as to the next steps in the process (Annexes 3). A number of papers had been prepared beforehand to assist the process. In particular, the document *Guidelines for nutritional policies* by Dr Natasha Vartepatova (Annex 4) was used as a guide for the group work.

## **Achievements of the project**

Following the workshop in October 1997, various lessons had been learned. The Government was clearly committed to the development of a food, nutrition and health policy but further efforts were needed to ensure that this commitment was sustained. In October there had been no clear consensus about what the content of the food and nutrition policy should be. Much debate centred around whether deficiencies of macro- and specific micro-nutrients are the main diet-related health priorities rather than identification of the major determinants of high mortality and morbidity. This debate has gone on in the Russian Federation for over ten years and it is clear that the contributions of international experts are needed to this debate so that the top health priorities can be identified and addressed as a matter of urgency.

One of the key achievements of the Workshop is that it provided the first step in helping to reach a consensus on what the content of a food and nutrition policy should be. The October workshop had shown that in order to make progress towards the development and implementation of a nutrition policy, it was necessary to build on a variety of links with health professionals from the regions. Some of these were frustrated by the lack of policy development, action and information about the main health problems which faced them daily. It is therefore likely that regional experts – of whom there exists a critical mass who, when combining their energy and force, can muster a great deal of power to influence change – will be motivated to take action to develop strategies needed at a regional level to promote health and prevent disease. Closer links are needed both with the regions and with the health professionals who are dealing with the main causes of mortality and morbidity. This, together with the growing emphasis on decentralization, strengthened the need to involve the regions.

All the objectives of the Workshop were achieved. The Ministry of Health recognizes it has a strong role to play in the development of food, nutrition and health policy in the Russian Federation. This was reinforced both in the formal presentation made at the Workshop and at subsequent informal meetings. The dietary physiological norms currently used in the Russian Federation were discussed and clarified by presenting the current disease and eating patterns in the country. This made it possible for the working groups to discuss potential ways of implementing disease prevention strategies, such as the development of dietary guidelines.

Intersectoral collaboration was fostered through group work and through the development of mutual respect and understanding, for example between representatives of the private sector and consumers' organizations and representatives of other government sectors as well as health. The groups addressed a number of topics related to the development of a framework for a food, nutrition and health policy, including the education and teaching of food culture in the Russian Federation. It is hoped that these sectors could be invited to participate in future workshops at regional level. Evidence of this cooperation can be seen in the examples of the group work in Annex 2.

Consumers' concerns were also covered, and three representatives of consumers' associations participated actively throughout the whole Workshop. They expressed their concern about and

willingness to continue to participate in the process initiated at the Workshop. Lively discussions took place concerning the import and labelling of foods during the presentation by the representative of the Anti-Monopolies Commission. There appear to be many examples of the public being cheated by misinformation and foods being sold to them through unethical practices. These practices will need to be monitored by sanitary epidemiology centres and the food law regulations enforced more effectively. This was the first time that many of those working in the health sector had had the opportunity to learn about consumer agencies, why they exist and how they might begin to work together.

Before the health sector can begin to strengthen alliances with consumers' associations, health professionals need to develop a better understanding of the role of these associations and identify how joint partnerships can strengthen the development of intersectoral health policies. The October workshop had identified a number of other factors which needed to be taken into account in the present Workshop, including:

- the need to nurture the development of civil society;
- the determinants of health in relation to the high premature mortality in the Russian Federation;
- the need for collaboration within the health sector as well as intersectorally;
- the need to work with consumer agencies;
- the need to inform and involve the agricultural and food sectors about current health issues;
- the need to strengthen alliances between different sectors involved with food, including environmental concerns;
- the need to strengthen Russian awareness of international initiatives regarding health promotion and disease prevention.

The need to nurture the development of civil society is clearly a long-term aim which cannot be achieved through one workshop. Nevertheless, the Workshop made a significant step towards achieving this. It was one of the first in the Russian Federation to promote the development of a health policy with intersectoral collaboration through participation from the voluntary, private and other governmental sectors in addition to health.

The determinants of premature mortality were presented by Dr Aileen Robertson from the WHO Regional Office for Europe, from the perspective of WHO, and by health professionals from the CINDI (Countrywide Integrated Noncommunicable Disease Intervention) programme network in the Russian Federation. It is clear that closer partnerships must be forged between health professionals in the regions, especially those involved in the CINDI network, and the Centre of Preventive Medicine in Moscow. Dr Robertson commented that it could be difficult to get agencies in the United Kingdom to work together because of political, cultural and economic differences. In the Russian Federation this can be even more challenging. The Workshop had, however, successfully brought together the Centre for Preventive Medicine and the National Centre for Preventive Medicine in the search for a consensus on the content of nutrition policies to prevent disease and promote health. It had also successfully brought together a range of health professionals from across the different regions of the Russian Federation.

It is often more straightforward to achieve a consensus within the health sector on the priorities for food safety and protection of public health through surveillance and enforcement of a food law than it is to agree priorities for nutrition targets. Nutrition targets are more complex and they

impinge on the work of a range of ministries and organizations. The Centre for Preventive Medicine and the National Centre for Preventive Medicine had achieved a consensus for collaboration within the health sector by agreeing on some common elements that should be included in the Russian Federation's food, nutrition and health policy.

The private sector was also represented at this workshop. The main food production business represented was Soya Ltd., Soybean Processors' Association in Krasnodar (Russian Federation). The General Director, Mr Alexander Podovedov, and one of his employees participated during the entire workshop and highlighted the views and perspective of the food industry. There are clearly many challenges and opportunities in working with the food industry and, in particular, in helping to achieve a common understanding of the key health problems and how best they can contribute to overall nutritional health and wellbeing in the Russian Federation.

Environmental issues were discussed during the workshop. Environmental issues are dealt with by the health professionals working in the sani-eped centres, which are part of the sanitary epidemiology system under the responsibility of the Ministry of Health. Often these health inspectors, or "hygienists", in the sani-eped stations are a major obstacle to promoting public health. For example Edict 55, passed in former soviet times and enforced by "hygienists", prevents babies staying together with their mothers in maternity hospitals. It was believed healthier to separate mothers from their babies to prevent transfer of infection to the babies. The practice of separating mothers and babies interferes with successful breastfeeding and makes babies more, not less, susceptible to infections.

"Hygienists" often issue warnings against breast-milk or eating locally grown vegetables because of the likelihood of high concentrations of PCBs in breast-milk or contaminants in the soil or atmosphere surrounding vegetables. These warnings may be issued without scientific evidence and, even more alarming, they may conflict with nutritional recommendations to promote breastfeeding and increase the production and consumption of vegetables. Discussion at the Workshop helped to clarify some of the issues and concerns related to environmental contamination and laid the foundations for a consensus.

Finally, one of the main achievements of the Workshop was to bring to the Russian Federation examples of successful international experience in health promotion and disease prevention. Both Finland and Wales are renowned for the food and nutrition policies they have developed in the context of their overall health policies. Moreover, the success of these strategies has been well documented and provides scientific evidence which proves the benefits of intersectoral policy development combined with community interventions. The participants were extremely interested to learn more about these practical examples and to try to understand how similar initiatives could be adapted to the Russian context.

The organizers felt that the Workshop exceeded their expectations in what was achieved both formally and informally. While it is difficult to measure formally some elements and their overall contribution to developing and implementing food, nutrition and health policies appropriate for the Russian Federation, there are a number of indicators supporting the view that the Workshop provided the first vital step to achieving both a consensus on the content of a food and nutrition policy and establishing a national and regional framework for such a policy for the Russian Federation. These indicators include:

- the organizers' subjective analyses
- participants' formal and informal evaluations (Annex 6)

- formal and informal feedback from the key partners (Annex 6)
- feedback from the group work (Annex 2)
- summary of discussions (Annex 3)
- plans for the next stage in the process (see Lessons Learnt and Problems Experienced (below) and Annex 3)
- working partnerships as evidenced throughout the three days.

## **Partnerships and sustainable developments**

Food safety and nutrition services are administered by the sanitary epidemiology system of the former USSR. To ensure sustainable policy development it is important that food policy issues relevant to both food safety and nutrition are considered as of equal relevance to public health. While the Workshop was running in Moscow the annual conference of the Sanitary Epidemiology System, attended by over 100 food hygienists, was taking place at the Ministry of Health. Dr Robertson was invited to give a presentation during this conference on the WHO perspective for a food, nutrition and health policy for the Russian Federation, and she reinforced the purpose and need for the Workshop.

During the Workshop additional support and commitment were sought from a number of important ministers. Visits were made to the Deputy Ministers of Health (Professor Onyshenko) and of Science and Technology (Mr V.A. Kniazhev) to enlist their support for the successful outcome of the Workshop. Follow-up letters have been sent to these ministries to remind them of their commitment to the development of a food, nutrition and health policy. The same strategy will be used in the regions: visits will be made to all senior policy-makers to secure their commitment to the process and to ensure support for the scientists and health professionals who carry out the work.

Sustainable development will largely be achieved through the commitment and dedication of the scientists and health professionals who signed up to the process during the Workshop. To encourage and motivate these people a number of activities were agreed:

- a core working group will be set up to determine a more detailed plan of action, consisting of representatives of the Moscow region;
- an estimated time schedule for activities was agreed and it was recommended that the first draft of the policy framework would be ready in December 1998;
- the need for guidance on how to carry out the preliminary needs assessment was highlighted; this would be clarified during May;
- two follow-up progress meetings would be held: the first in June in Copenhagen, and the second on 1–3 October in one of the participating regions (probably Chelyabinsk) to assess progress.

The Workshop was the start of an important initiative and one of the challenges now will be to secure funds to maintain the momentum. Potential sources are being sought, including embassies in the Russian Federation, the Soros Foundation, WHO and other United Nations agencies, and through international collaboration with bilateral agencies and with EU initiatives such as TACIS.

The completed framework for the Russian Federation's regional food, nutrition and health policies will provide good practice models for other regions, in addition to supporting the

implementation of national policies and activities. This way of working could provide a good model for others wishing to invest in the Russian Federation in a similar way.

## **Roles of partners**

The main partners in this initiative were:

- the Russian Institute of Nutrition
- the Centre for Preventive Medicine (including the WHO CINDI network)
- Health Promotion Wales
- the World Health Organization (WHO).

### ***Health Promotion Wales***

Health Promotion Wales is an acknowledged leader in the development of health promotion and health education. Established in 1987 as part of the National Health Service (NHS), its roots were in Heartbeat Wales, the pioneering initiative to combat heart disease in Wales. Health Promotion Wales works closely with organizations in the NHS, and with local government, schools and colleges, charities, voluntary organizations, and business and industry. Partnership is a central feature of its approach.

Health Promotion Wales is a WHO collaborating centre for health promotion and the development of health education. It has an international reputation for innovative approaches to health promotion and has undertaken a range of contracts on behalf of the European Commission and WHO. Health Promotion Wales' library is a WHO documentation centre.

Health Promotion Wales was able to bring this experience to the Russian Federation and through the Workshop to support the development of partnerships and ultimately the development of a food, nutrition and health policy.

### ***World Health Organization***

WHO has a long-standing commitment to assist the Russian Federation to develop health policies in line with the health for all concept, based on intersectoral policy development, such as the development of food, nutrition and health policies.

The Barents/WHO project in north-west Russia on infant feeding links the Know-How partners from the Centre for Preventive Medicine and National Centre for Preventive Medicine in Moscow with the Arkhangelsk region. Another new WHO initiative is about to start in north-west Russia, with the involvement of all the Know-How partners, to address issues concerning child and maternal nutrition and.

WHO and CINDI are in the process of developing CINDI dietary guidelines. It is hoped that these will be available later in 1998. They will provide a useful framework to support the regions in the Russian Federation in the development of food, nutrition and health policies, and can be adapted to their own regional needs. Through WHO, the Russian Federation has access to many books, documents, and educational and training materials to help strengthen its capacity in health promotion and disease prevention. Small amounts of financial support are available through WHO country funds.

### ***National Centre for Preventive Medicine***

The National Centre for Preventive Medicine is a leading institution in health promotion and disease prevention in the Russian Federation. Its expertise has grown from cardiovascular epidemiology and prevention.

During the previous two years the Centre had developed and published two documents: *Towards a healthy Russia, policy for health promotion and diseases prevention: focus on major noncommunicable diseases* and *Policies and strategies for the prevention of cardiovascular and other noncommunicable diseases within the context of public health care reforms in Russia*. These documents analysed the health situation and existing issues in this field and presented the main strategies for developing and implementing national and regional policies for disease prevention and health promotion in the Russian Federation.

The National Centre for Preventive Medicine works closely with governmental, nongovernmental and professional organizations at national and regional level. The Centre is a coordinator of the Russian WHO CINDI network of regions collaborating in the field of health promotion and disease prevention. Twelve regions are official members of CINDI-Russia and another 10 have applied to become members. The main partners in the regions are the regional governments, medical universities, institutions for preventive activities, and primary health care educational establishments.

The regions mainly use their own resources for developing preventive activities. The National Centre for Preventive Medicine provides supervision, expertise, and coordination of training as well as international links with foreign leading specialists and organizations in the field of health promotion and disease prevention.

### ***Institute of Nutrition***

The Institute of Nutrition is a government medical organization under the Russian Academy of Medical Sciences. It was established in 1930, since when it has been the leading research centre dealing with the development of theoretical and practical principles of nutrition science.

The Institute consists of 30 laboratories and a 200-bed clinic. There are approximately 150 research workers, including 30 professors and doctors of science. Over 100 of the researchers have Ph.Ds. The scientists at the Institute are permanent or temporary advisers of WHO and other international organizations. The Institute collaborates with the Ministries of Health, Agriculture, Science and Labour in the Russian Federation, and with international agencies such as WHO, the United Nations Environmental Programme and scientific institutions in Finland, France, Italy, Poland, Switzerland, China and the United States.

The main trends in the Institute's research are:

- study of the metabolism of nutrients in the human body and determination of requirements for energy and nutrients;
- studies in nutrition epidemiology (food consumption, nutrient intake, nutrition status);
- studies of food quality (chemical composition of foods and creation of food composition tables) and food safety (development of analytical methods for detection, identification and quantitative definition of contaminants);
- development of recommendations for healthy nutrition in the population.

## **Complementarity of the project**

International donors have traditionally supported the curative and acute health services in the Russian Federation rather than supporting strategies which focus more on health promotion and disease prevention. This appears to be changing, however, and examples now exist such as the 4 million ECU awarded by the EU through the TACIS scheme. Two of the consultants working on the TACIS project in the Russian Federation participated in the Workshop and additional meetings were held to discuss how the TACIS project could be linked to the nutrition policy development process initiated by the Workshop. The Soros Foundation was supporting health promotion and disease prevention and the Centre for Preventive Medicine had had several meetings to discuss possible future funding opportunities.

WHO had arranged a Health for All policy conference on 6 May with the Ministry of Health in Moscow to discuss the health for all concept and the need for intersectoral policy development. Professor Oganov, Director of the Centre for Preventive Medicine (one of the partners), had been invited to speak and to inform the Ministry of Health about the workshop, which was one of the first intersectoral health policy workshops to be held in the Russian Federation.

On 29–30 September, 1998 the International Life Sciences Centre for Preventive Medicine (ILSI) was arranging a conference in Moscow entitled “Food Safety and Nutrition Policy: Developments in Safety Assessment and Nutrition Research”. This was being organized in collaboration with the Centre for Preventive Medicine, and Dr Robertson would ensure that the policy development was described in detail.

## **Lessons learnt and problems experienced**

Some of the problems experienced during this initiative included the following.

- Everything from administration to communication takes longer when developing partnerships in the Russian Federation. It is advisable to estimate how long these would take in the United Kingdom and double this in order to get a realistic time frame and deadline for projects.
- In the Russian Federation there is no basic understanding of what is meant by the word “policy”. Previously this word was reserved by the Communist Party and there was only one policy, namely that of the communist regime. This is one of the reasons why there is no basic understanding of the process needed to reach a consensus and so develop common policies which have been agreed by all the key stakeholders. There is no clear understanding that policies cannot be implemented without the commitment of all the stakeholders.
- It is very difficult to develop coalitions and partnerships between Russian institutes which have traditionally competed for funds and not shared information with each other.
- Clearer details on the procedure to gain a visa to the Russian Federation would be useful. Some difficulty in understanding the procedure and requirements for a visa was experienced with different agencies saying different things. Letters of approval and support from partner agencies were difficult to coordinate with the embassies.
- Differences between the financial systems and means of transferring money from the United Kingdom to partner agencies have created major difficulties, and individuals have had to use their own personal resources in the first instance. It appears to be impossible to transfer money to partner agencies and, as a result, money has had to be given to personal contacts at



the agencies so that they could open separate accounts. This is against the standing financial instructions of Health Promotion Wales and similar agencies, which made this process very difficult. Further advice would be welcomed as to how others transfer money in similar circumstances.

- Between the time of the original application and the time of approval the needs had changed and the work had to be modified accordingly. Account should be taken of this.

### ***Other information***

General lessons concerning partnerships in the Russian Federation include the following.

- The Russian partners need good functioning communication technology such as e-mail, fax and telephones.
- Financial systems need to be in place for the Russian partners. They need funds to cover expenses beforehand as they do not have access to cash and are otherwise forced to use their private savings.
- It is very important to have good English-speaking Russian partners and to develop a trusting relationship with them.
- It was very useful to be able to link up with existing networks and communication systems; for example, the ability to access the regions through the CINDI network saved an enormous amount of time and effort.
- An understanding of the differences in the social, cultural and political climates in the Russian Federation which will inevitably have an impact on any initiative is only gained through personal experience of the conditions there. The partners from outside the country felt that as a result of their visit they were far better equipped to take forward any further work in the Russian Federation, and welcomed the challenges that this presented.

## **Developments of the health sector**

In general the health sector in the Russian Federation is over-medicalized and this is reflected in their outlook and the way that services are organized and supported. Most funding goes to acute or curative medicine.

Health promotion and disease prevention are neglected areas and generally given very little priority. It is important to encourage international donors to support preventive services and place greater emphasis on supporting strategies focused on health, health promotion and disease prevention. Examples exist such as the 4 million ECU award by the EU through the TACIS scheme and this Workshop funded by the Know-How Fund.

Most health professionals working in Russian acute care and curative medicine resist change since they fear losing their jobs (e.g. falling birth rates reduce the need for gynaecologists and obstetricians). Funding therefore needs to go into policy development and implementation which would include a more structured and integrated approach to issues such as retraining schemes in the area of health promotion and disease prevention.

## The way forward

Participants committed themselves to take this initiative forward in partnership with key agencies at national and regional levels. The way forward was discussed and further details are outlined in Annex 3. In summary, the following is an outline of the key actions and time frames agreed.

- A follow-up meeting will be held after a CINDI network meeting in June. This will review progress to date, discuss any potential problems and help maintain the momentum of the initiative.
- The first draft of the regional situation analysis and needs assessment will be submitted by the end of September. This will list available data sets which can be used for policy development and identify information gaps at regional level.
- Another meeting of key partners will be held on 1–2 October, 1998 to assess progress, since it is anticipated that a first draft of the food, nutrition and health policy framework will be ready by December.
- Further support is required to continue the consultation process and to facilitate the production of and consultation on the first draft of the policy framework. Additional funds will be needed to fill the information gaps identified over the coming months. This will probably require that additional surveys are carried out.

An exciting process has started and over the next 12 months there will be dramatic developments in the understanding and adoption of policies at both national and regional levels in support of health. We hope that the Know-How Fund will recognize the importance of the progress and commitment made to date by all the partners involved and continue to support the innovative work initiated at the Workshop.

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*Annex 1*

**PROGRAMME**

**Tuesday 21 April**

*Chairpersons: I. Glasunov, A. Robertson*

- |               |                                                                                                                                                                         |
|---------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| 13.00 – 13.20 | Opening. Welcome from Ministry of Health, Ministry of Science, National Centre for Preventive Medicine, Centre of Preventive Medicine, WHO EURO, Health Promotion Wales |
| 13.20 – 13.35 | Presentation of participants                                                                                                                                            |
| 13.35 – 13.50 | Aims and objectives of the workshop. <i>I. Glasunov</i>                                                                                                                 |
| 13.50 – 14.10 | Discussion                                                                                                                                                              |
| 14.10 – 14.25 | WHO nutrition policy. Perspectives of nutrition policy development in the Russian Federation, particularly at regional level. <i>A. Robertson</i>                       |
| 14.25 – 14.35 | Discussion                                                                                                                                                              |
| 14.35 – 14.50 | Experience in policy development for health promotion and diseases prevention. <i>R. Oganov</i>                                                                         |
| 14.50 – 15.00 | Discussion                                                                                                                                                              |
| 15.00 – 15.15 | Break                                                                                                                                                                   |
| 15.15 – 15.30 | Nutritional status of the Russian population. <i>A. Baturin</i>                                                                                                         |
| 15.30 – 15.40 | Discussion                                                                                                                                                              |
| 15.40 – 15.55 | The Welsh experience in public health policy development at regional level. <i>H. Howson</i>                                                                            |
| 15.55 – 16.05 | Discussion                                                                                                                                                              |
| 16.05 – 16.15 | Nutrition policy from the point of view of an ordinary citizen. <i>I. Vohlonen</i>                                                                                      |
| 16.15 – 16.30 | Role of Ministry of Health, State Sanitary Inspection in policy development at regional level. <i>A. Petukhov, G. Tkachenko</i>                                         |
| 16.30 – 16.45 | State anti-monopoly committee, opportunities at regional level. <i>Y. Kokovikhin</i>                                                                                    |
| 16.45 – 17.00 | Discussion                                                                                                                                                              |
| 17.00         | Social programme                                                                                                                                                        |

**Wednesday 22 April**

*Chairpersons: R. Oganov, H. Howson*

- |               |                                                                                                                                    |
|---------------|------------------------------------------------------------------------------------------------------------------------------------|
| 09.30 – 09.45 | Role of nongovernmental consumer organizations in nutrition and food policy development in regions. <i>D. Yanin, E. Kuznetsova</i> |
| 09.45 – 10.00 | Discussion                                                                                                                         |
| 10.00 – 10.10 | Education of different population groups on healthy nutrition. <i>Y. Krupnov</i>                                                   |
| 10.10 – 10.20 | Discussion                                                                                                                         |
| 10.20 – 10.35 | Nutrition policy development in Krasnodar region. <i>M.N. Danchin, A.N. Nikolaevich</i>                                            |
| 10.35 – 10.50 | Role of nongovernmental sector in nutrition and food industry in regions. <i>A. Podobedov</i>                                      |
| 10.50 – 11.00 | Discussion                                                                                                                         |
| 11.00 – 11.15 | Break                                                                                                                              |
| 11.15 – 11.30 | Nutrition policy development in Tyumen region. <i>S.I. Mataev</i>                                                                  |

- 11.30 – 11.45 Nutrition policy development in Chelyabinsk region. *E.G. Volkova, I.F. Kalev, R.P. Makashina*
- 11.45 – 11.55 An example of developing of nutrition policy: breastfeeding promotion programme in Electrostal. *V.A. Chekunov, L.I. Romanchuk*
- 11.55 – 12.10 Discussion
- 12.10 – 12.30 Main directions and priorities in nutrition policy development at regional level. Its aims and objectives. (Drafted format of the policy document, jointly prepared by the National Research Centre for Preventive Medicine and Institute of Nutrition). *N. Vartapetova, A. Baturin*
- 12.30 – 13.00 Discussion
- 13.00 – 14.00 Lunch
- 14.00 – 15.45 Work in regional groups on:
- objectives of nutrition policy at regional level
  - issues
  - strategies
  - priorities
  - needs assessment
  - directions in policy development
  - monitoring and evaluation
- 15.45 – 16.00 Break
- 16.00 – 18.00 Healthy Dinner prepared by the Soy-bean Corporation

### **Thursday 23 April**

Chairpersons: *A. Baturin, O. Kalev*

- 09.30 – 11.15 Continuation of work in the regional groups
- 11.15 – 11.30 Break
- 11.30 – 12.30 Presentations of results of discussion in regional groups. Summary of presentations
- 12.30 – 13.00 Discussion
- 13.00 – 14.00 Lunch
- 14.00 – 14.30 Agreement on:
- format of the policy document
  - creation of an working group
  - further activities
- 14.30 – 15.00 Closing

## *Annex 2*

### SUMMARY OF FEEDBACK FROM WORKING GROUPS

A preliminary document was produced and presented to the groups to help focus discussions and achieve the task quickly.

Groups were formulated primarily on the basis of regional units but with representatives from a range of agencies and organizations.

The following includes samples of feedback from two groups.

#### **GROUP 1 (see sheets from 1<sup>st</sup> group)**

Group 1 included participants from the Chelyabinsk Centre of Preventive Medicine and the Ural State Academy of Additional Training. They presented the following plan and framework for drawing up a regional policy:

- to analyse existing regional data on nutrition and health status with emphasis on CVDs, cancer, osteoporosis, obesity, anaemia, iodine deficiency, children's health status;
- needs assessment; barriers for introducing healthy diet;
- target groups: babies (breastfeeding), children and adolescents, socially disadvantaged groups;
- positive examples of changing diet and improving health status;
- the importance of calories, fat, carbohydrates, fibres, vitamins for health status;

#### *Main strategies*

- Capacity- and coalition-building, involving partners from different sectors
- Regional legislation on food and nutrition
- Marketing a healthy diet at the regional level, using the regional mass media, introducing health promoting campaigns
- Using different channels for public education (shops, mass catering, schools, kindergartens, work-places, universities, etc.)
- Professional education on public health nutrition. Organizing federal and regional training centres. Developing national and regional guidelines in accordance with international recommendations
- Regional agricultural policy with emphasis on developing regional facilities for food production
- Elaborating monitoring and evaluation tools
- International collaboration on food and nutrition policy and healthy diet

#### *Additional comments*

- An additional study to highlight local needs in the region should be undertaken by December 1998.
- A glossary of terms should be included.
- Reference to should be made physiological needs and information should be translated into foods.
- The prepared document was useful.

- Short-, medium- and long-term time scales for implementation should be developed along with clear aims and objectives for each specific activity.
- More specific information may be needed to help define the roles and responsibilities of different groups such as caterers, general practitioners and food producers. Some discussion took place regarding how best this document could be taken forward. Wider coalitions should be developed locally with representatives from a variety of sectoral group and industry.

## **GROUP 2**

### *1. Determine needs*

- Demographic and geographical data
- Social and economic parameters
- Determine factors such as unemployment and issues such as the needs of older people
- Local food production, imported foods
- Mass catering outlets – numbers who eat where, who what and when
- Major source of financing the region
- Attitudes of different sectors towards initiative.

### *2. Potential partners*

- Education
- Health care
- Sanitary inspectors
- Mass media
- Insurance

### *3. Priority groups in the population*

- Population – as a whole implement between 5 and 7 years
- Adolescents
- Doctors
- Pregnant and nursing mothers

### *4. Data needed*

- Dietary habits, what, why , how they eat
- Income levels and percentage of the budget spent on food
- Attitudes towards foods
- Accessibility and availability of foods

### *5. Aims and objectives*

Aim: To provide health among the population

Objective 1: To educate the population in the principles of healthy nutrition

Objective 2: To educate new professionals, producers and suppliers of food.

### *6. Strategies*

- Construct an intersectoral working group
- Evaluation needs – continually in a spiral
- Stimulate domestic producers to produce food – there should be economic incentives at a regional level. Some incentives built in on a cost basis.
- Supplementing groups in work

- Work out sanitary and other requirements
- Marketing and mass media and direction information for the population.
- Education programmes for health professionals, sanitary inspection committee, teachers, etc.
- Social public organizations:
  - education programmes in schools
  - hot lines for schoolchildren
- Set up different working groups for each age group
- Education of professionals:
  - teachers
  - medical workers
  - others
- Trading and labelling – need to have education sessions with professionals in this field



### *Annex 3*

## SUMMARY OF DISCUSSIONS

### **General**

Groups approached the work in different ways but agreed on a number of common points.

- The importance of nutrition policy.
- The need to strengthen the interrelationship between nutrition and health.
- Evaluation of needs should be from existing data and the need for additional information studies.
- The need for partnerships and intersectoral collaboration at regional and national levels.
- The need for targets or priority groups of the population.
- Application of various strategies: education strategies, marketing healthy nutrition, legislation.
- Information should be clear, honest and truthful.
- The specific guidelines being developed by the CINDI network should be considered further in the light of these discussions.
- Recommendations for individuals on consumption levels should be included.
- Evaluation of the process should be included as well as outcome evaluation.
- Time limits of the strategy and the document should be highlighted. Five years is a realistic time frame for the overall strategy.
- The document should provide guidelines for action as well as a framework. It should be directed at a wide range of individuals and not just health professionals. Further information should be collected from other partners.
- This is the finalization of the first stage. Consultation on the detailed formulation of the document should be extended into the autumn so that a final document is prepared by the end of the year.
- A working group should be set up to help determine a more detailed plan of action.
- It was important to clarify how best to translate the agreed guidelines with information for the public. Different models were discussed, and the pyramid model was thought to be the most relevant in the Russian Federation.

### **Format of the document on food, nutrition and health policy**

It was agreed that the format of the proposal put forward by group 2 should be adapted.

### **Working groups**

It was suggested that a national and a regional working group should be established.

#### *National working group*

All participants in the workshop were proposed as members of the group, plus representatives of the Ministry of Science and the sanitary inspectors. The Commissioners' Association was also keen to be involved and will become part of this group. The State Statistical Committee would also be a useful addition but no formal approach had yet been made. It was also suggested that a representative from the

Ministry of Agriculture should be included. The list of members is not a closed list and should remain open but representative.

#### *Regional working group*

The regions would decide on membership of the regional group.

#### **Future activities**

It was agreed to allow the participants time to think a little when they return home and send any further comments or feedback on the discussions to date, e.g. on the structure of documents and how and when to work out the details. These should be approved by local groups and received before June.

A small working group (editorial board) in Moscow will aim to draw together an outline document on the basis of the information presented and discussions at the workshop.

#### *Timetable*

|                           |             |
|---------------------------|-------------|
| Workshop                  | April       |
| Working plan              | May         |
| Regional needs assessment | June/August |
| Regional meeting          | October     |
| First draft document      | December    |

Further guidelines as a general consensus or a protocol were needed to ensure similar needs assessment practices are adopted by the regions.

*Annex 4*

## GUIDELINES FOR REGIONAL NUTRITIONAL POLICIES

Dr Natasha Vartepatova

1. Need to draw up a nutrition policy

2. Nutrition and health

- CVD – cholesterol and other factors
- cancer
- osteoporosis
- diabetes
- obesity
- anaemia (iron deficiency)
- breastfeeding
- nutrition disorders in children

Need to consider primarily over-eating and its problems – secondary issues of efficiency

3. Situation analysis

4. Interrelation between the nutrition of the population and effects on health

- fat
- carbohydrates
- sugar
- vitamins
- minerals

5. Possibility of changing the nutrition style of the population

6. Regional and national experience in the field of national policy

7. Partnerships

8. Evaluation of the needs of different populations

9. Jurisdiction – legal aspects

10. Marketing and promotion of healthy healing nutrition in various population groups – mass media

11. Who, how and what services are to work in the population

12. Regional opportunities for educating groups of people

13. Education of professionals

14. Consumers: Do they exist? How can we support them?

15. Priority groups in the population depend on the region

- children under 7 years of age
- school children and adolescents
- pregnant and lactating women

16. Agricultural production policy

17. Production and processing of food

18. Trading laws and labelling of foods

19. Mass catering

20. Monitoring and evaluation programme

## Annex 5

### EVALUATIONS AND CORRESPONDENCE FROM PARTNERS

**Total number of evaluation forms: 18**

| <i>Questions</i>                                                                                      | <i>Answers</i>                                                                                                             |
|-------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------|
| 1. Do you think that there should be a clear food, nutrition and health policy at a regional level?   | Yes – 18 (100%)                                                                                                            |
| 2. Do you think that there should be a clear food, nutrition and health policy at a national level?   | Yes – 18 (100%)                                                                                                            |
| 3. To both, how should they best be linked                                                            | Yes – 11 (61%)                                                                                                             |
| • joint food, nutrition, health committee                                                             | Yes – 9 (50%)                                                                                                              |
| • joint agreed targets                                                                                | Yes – 8 (44%)                                                                                                              |
| • clear areas of responsibility                                                                       | Yes – 16 (89%)                                                                                                             |
| • effective communicative strategy                                                                    | Yes – 15 (83%)                                                                                                             |
| • between regional and national levels                                                                |                                                                                                                            |
| • annual meetings to review progress and future directions                                            |                                                                                                                            |
| 4. How adequate was the information provided by the organizers before the workshops?                  | Completely adequate – 18 (100%)                                                                                            |
| 5. Did you find the preparation of background papers enhanced your participation during the workshop? | Very much – 17 (95%)                                                                                                       |
| 6. Have the group discussions been useful?                                                            | Very useful + useful – 18 (100%)                                                                                           |
| 7. How useful the support materials made available at the workshop could be in the future?            | Very useful + useful – 16 (89%)                                                                                            |
| 8. Did you find the venue and facilities:                                                             |                                                                                                                            |
| • inadequate                                                                                          | 0                                                                                                                          |
| • adequate                                                                                            | 0                                                                                                                          |
| • completely adequate                                                                                 | 18 (100%)                                                                                                                  |
| 9. Please indicate your future training needs in support of policy development                        | International training courses, workshops at the national Research Centre for Preventive Medicine (Moscow), WHO guidelines |

**COPY**

**Ministry of Public Health of Russian Federation  
NATIONAL CENTRE FOR PREVENTIVE MEDICINE  
Petroverigsky str., 10 Moscow  
Russia 101953  
Tel: 7 (095) 928 21 37  
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To: Ms Christine Forrester  
Manager  
Health Sector – Small Partnership Scheme (Russia)  
Know-How Fund  
Charities Aid Foundation  
114/118 Southampton Row  
London WC21B 5AA

18 May, 1998

Dear Ms Forrester

On behalf of the national Research Centre for Preventative Medicine I would like to express our gratitude to the Know-How fund support of the healthy nutrition policy development in Russia.

The Workshop “Public Healthy Nutrition Policy Development at National and Regional Levels in Russia” jointly organized by the National Research Centre for Preventive Medicine, Institute of Preventive Medicine, Moscow Nutrition, Know-How Fund, Health Promotion Wales and WHO/EURO took place in Moscow from the 20<sup>th</sup> to the 24<sup>th</sup> of April, 1998. This workshop has given a unique possibility to assemble representatives of various disciplines and sectors, governmental and nongovernmental organizations and to discuss a range of issues relevant to formulation of comprehensive health nutritional policy, especially regional one in Russia.

It was especially beneficial to us that Dr Helen Howson from Health Promotion Wales participated in the workshop on behalf of Know-How Fund. Her contribution was outstanding and we would like to hope that our contact with her can have continuation.

The report on the workshop is in the preparation and will be submitted to you in due time. The purpose of my letter is just to thank you for making this important event possible, the event which is sure to have its follow up and impact in my country.

Looking forward to continue our fruitful collaboration.

I attach also the letter from Prof. Emilia G. Volkova, a participant from the Ural Medical Academy of Additional Education we have received some time ago.

With kindest regards,

(signed) I Glasunov

I.S. Glasunov, M.D.  
Professor and Head,  
Department of Policy and Strategy Development in Diseases Prevention and Health Promotion

CC: Dr Helen Howson, Health Promotion Wales.

## **COPY**

**To: National Research Centre  
for Preventive Medicine  
Know-How Fund  
Health Promotion Wales  
WHO European Office**

**From: Prof. Emilia G Volkova  
Ural Medical Academy of  
Additional Education  
Chelyabinsk, Russia**

Dear colleagues,

On behalf of Chelyabinsk participants of the seminar “Public Healthy Nutrition Policy Development at National and Regional levels in Russia”, I would like to thank cordially all organizers for the excellent documented seminar. It is the first Russian experience on working out a nutrition policy and intersector partnership organization. This experience will be the new stimulus for increasing of efforts in our region. It will be useful and necessary school for all who take part in the programme of NCD prevention.

Once more thank you very much. We are looking forward to our fruitful collaboration

Sincerely Yours.

(signed) E Volkova

Professor *Emilia G. Volkova*  
Chief cardiologist of Chelyabinsk region

*Annex 6*

**PARTICIPANTS**

| <i>Name</i>                                                                                                                                                                                                       | <i>Tel/Fax/E-mail</i>                                                                                                                                     |
|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Russian Federation</b>                                                                                                                                                                                         |                                                                                                                                                           |
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| Irina M. Gavrilenko                                                                                                                                                                                               |                                                                                                                                                           |
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*Annex 7*

## RESOURCE DOCUMENTS

|    | <b>Title</b>                                                                                                                                  | <b>Author</b>                                                                                          | <b>Language</b> |
|----|-----------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------|-----------------|
| 1  | Reversibility of rise in Russian mortality rates                                                                                              | Kromhout, D. et al. National Institute of Public Health and Environment, Bilthoven, Netherlands        | R, E            |
| 2  | Hygiene in food-service and mass catering establishments                                                                                      | WHO Headquarters, Geneva                                                                               | R, E            |
| 3  | Eating for health: a diet action plan (Draft in Russian)                                                                                      | The Scottish Office, United Kingdom                                                                    | R, E            |
| 4  | Dietary reference values                                                                                                                      | Department. of Health, London, United Kingdom                                                          | R, E            |
| 5  | Diet, nutrition and the prevention of chronic diseases                                                                                        | Geneva, World Health Organization, 1990 (WHO Technical Report Series No. 797).                         | R, E            |
| 6  | Health promotion challenges for countries of the former Soviet Union: results from collaboration between Estonia, Russian Karelia and Finland | Puska, P. Division of Health and Chronic Diseases, National Public Health Institute, Helsinki, Finland | R               |
| 7  | Social marketing and public health intervention                                                                                               | Lefebvre, R.C. & Flora, J.A. USA                                                                       | R, E            |
| 8  | Measuring obesity – classification and description of anthropometric data. Report on WHO Consultation, Warsaw, October 1987                   | WHO                                                                                                    | R, E            |
| 9  | Regional conference on elimination of IDD in CCEE, CIS, and Baltics, Munich, September 1997: Priorities for eliminating IDD in CCEE and NIS   | Robertson, A. Copenhagen, WHO Regional Office for Europe                                               | R, E            |
| 10 | Protein reference values in the Russian Federation, 1992                                                                                      | Popkin, et al.                                                                                         | R, E            |
| 11 | Healthy eating during pregnancy and lactation (training module)                                                                               | WHO                                                                                                    | R, E            |
| 12 | Food pyramids                                                                                                                                 |                                                                                                        | R, E            |
| 13 | Healthy living – advice re tobacco, alcohol and eating                                                                                        | BBC and WHO                                                                                            | E               |
| 14 | Regional Nutrition Programme Advisory Group                                                                                                   | Copenhagen 28-30 August 1995                                                                           |                 |
| 15 | Food health policies – the company approach. A paper prepared for the HEA                                                                     | Jennifer Poulter August 1990                                                                           |                 |
| 17 | Food and health policy for Brent: report of an ad hoc Working Group,                                                                          | Brent Health District, United Kingdom                                                                  |                 |
| 18 | Food and nutrition policy in Europe                                                                                                           | Report on WHO Conference Budapest October 1990                                                         |                 |
| 19 | Guidelines on formulating a food policy                                                                                                       | Yorkshire Regional Health Authority, United Kingdom                                                    |                 |
| 20 | Nutrition policy in WHO European Member States. Progress report following the 1992 International Conference on Nutrition                      | WHO June 1995                                                                                          |                 |
| 21 | District food and health policy (Promoting health in Stockport)                                                                               | Stockport Health Authority, United Kingdom                                                             |                 |
| 22 | UK Study of District Health Authority and Health Board Food Health Policies (England, Wales, Scotland and                                     | Liz Gibson & Joyce Kallevik HEA                                                                        |                 |

|    | <b>Title</b>                                                                                                                  | <b>Author</b>                                        | <b>Language</b> |
|----|-------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------|-----------------|
|    | Northern Ireland)                                                                                                             |                                                      |                 |
| 23 | Food policy and the consumer                                                                                                  | National Consumer Council                            |                 |
| 24 | Food law: fear of the future? An analytical report on the food regulatory process and Government health and nutrition policy. | London Public Policy Consultant, United Kingdom      |                 |
| 25 | Results of the 1986 Food Health Policy Survey UK National Study                                                               | Lizz Gibson & Philippa Champion HEA                  |                 |
| 26 | Low income, food, nutrition and health: strategies for improvement                                                            | Low Income Project Team for the Nutrition Task Force |                 |
| 27 | Dietary reference value guide                                                                                                 | Department of Health, United Kingdom                 |                 |
| 28 | The history, implementation and evaluation of the food and health policy Jan 1983 to Dec 1986                                 | Bloomsbury Health Authority, United Kingdom          |                 |
| 29 | Health for all in Wales Part C - Strategic Directors for the Health Promotion Authority                                       | Health Promotion Wales, United Kingdom               |                 |
| 30 | Take heart – Consultative document on the development of community based heart health initiatives within Wales                | Health Promotion Wales, United Kingdom               |                 |
| 31 | Health for all in Wales – The way forward                                                                                     | Health Promotion Wales, United Kingdom               |                 |